

Options, Florida FFS GME program

Summary

Florida GME payments are not clearly tied to Medicaid costs in most sections of the approved GME language. The first paragraph of the GME section in Section VIII on p. 41 specifies that the “Medicaid Residency Program is established to improve the quality of care and access to care for Medicaid recipients, expand graduate medical education on an equitable basis, and increase the supply of highly trained physicians statewide. AHCA shall make payments to hospitals and qualifying institutions for graduate medical education associated with the Medicaid program.” As originally written in FL-18-0005, the statement applied to the entire GME section. As currently written, it seems to apply only to the payments under Section VIII(A). Florida’s state plan has issues of comprehensiveness with some of the approved payments, and all the payments should be more clearly tied to delivery of Medicaid services.

Current State Plan GME Issues (FL-24-24-0008)

FL-24-0008 is currently off the clock and contains several GME-specific updates:

- Slots for Doctors program (GME payments of \$100,000 per newly created resident position in a physician specialty in statewide supply-and-demand deficit, and \$150,000 per newly created resident position in Behavioral Health Teaching Hospitals)
- Primary Care Graduate Medical Education program (expands \$150,000 payments per GME FTEs in Primary Care from Medicaid Region 1 to Regions 1 and 2; adds \$175,00 payments per GME FTEs in Primary Care in Region 4)
- Documents the annual pool amounts for each of the GME payment programs in Section VIII(B)-(H): (Graduate Medical Education Startup Bonus Program, Primary Care Graduate Medical Education, High Tertiary Statutory Teaching Graduate Medical Education, Mental Health Graduate Medical Education, Adult and Child Psychiatry for Federally Qualified Health Centers, Slots for Doctors, and Slots for Doctors: Behavioral Health)
- Revises the pages to document the maximum pool amounts for each type of GME payment

Potential FFS GME Methodologies

Florida has six different types of direct GME payments in Sections (A) through (F), and this SPA would add two more, (G) Slots for Doctors, and (H) Slots for Doctors – Behavioral Health. The first of these payments, Section VIII(A) Medicaid Residency Program payments, is calculated using a formula tied to Medicaid services and allocated based on a hospital’s percentage of the total statewide FTE residents.

The other GME payments, in Section VIII(B)-(H), are paid a set per-resident amount (\$100k, \$125k, \$150k, or \$200k) up to a maximum total pool amount for each type of payment. As written, these payments aren’t allocated to the costs to Medicaid of the different resident positions.

DRR has summarized four methodologies for reference in further developing Florida’s GME program:

1. Revise Florida’s existing language (*see below, Option 1*). Florida could clarify its existing state plan language to improve the comprehensiveness and economy/efficiency of its current GME payment language.
2. North Carolina’s FFS GME Methodology with Medicare Comparison (follows Medicare principles more comprehensively)

3. Summary of Nebraska FFS GME Methodology (straightforward but may not adhere to complex populations)
4. Illinois FFS GME Methodology Summary.docx (adheres to more complex populations)

Option 1. Revise Florida's existing language.

Florida's state plan language pays hospitals a set amount per resident, up to a maximum pool amount for each payment, for each of the direct GME payments in Section VIII(B)-(H). Florida's latest revision to the SPA pages documents the maximum pool amounts for each type of payment. As written, the per-resident payments aren't allocated to the costs to Medicaid of the different resident positions within the hospitals.

For Florida to meet the requirements of economy and efficiency, the state would need to clarify that the language in Section VIII(A) detailing payments to hospitals "for graduate medical education associated with the Medicaid program" applies to all GME payments to hospitals made in Section VIII, not merely to those in par. (A). Florida would also need to demonstrate that the individual types of direct GME per-resident payments are equal to or less than the cost to Medicaid for these services. The approach might specify, for each GME payment, that the per-resident payment amount is the hospital-specific cost to Medicaid per resident FTE but with a limit set at the per-resident amounts included for each GME resident payment program (for (G) Slots for Doctors, for example, a maximum of \$100,000).

(Because each payment program also has a specific pool amount, the state would also need to include language reducing the per-FTE payments proportionally if the number of eligible FTEs multiplied by the maximum per-resident amount in a given year exceeds the amount of the state's annual pool. This language is included in the latest revision for Slots for Doctors, but not in all the GME payments.)

In support of the economy and efficiency of the Slots for Doctors payments, Florida stated that the payment per resident amounts were determined through conversation with the Teaching Hospital administrators, the Safety Net Hospital Alliance of Florida, and were based on the results of the HIS Markit: Florida Statewide and Regional Physician Workforce Analysis: 2019 to 2035 published in December of 2021. Florida also submitted the results of two analyses in 2014 and 2016 showing the costs to train residents is between \$179,353 per year and \$454,009 per year. Per Florida, "Assuming the trend remained constant over the time frame 2016-2025, the average annual cost per resident would be significantly higher than \$179,353." If the state demonstrates that the GME payment per resident in each hospital is less than the cost per resident to Medicaid (perhaps following a formula similar to that used by North Carolina), it could demonstrate the economy and efficiency of the payments.

Option 2. Change methodologies.

Change the direct GME methodology, following one of the three suggestions attached or a new approach. Links to State Plan pages are included.

- North Carolina, pages 8b-8c
- Nebraska, page 7
- Illinois, page 135

Calculation of Medical Education Costs

Calculation of Direct Medical Education Cost Payments:

Direct Medical Education (DME) payments effective October 1, 2009 are based on Nebraska hospital-specific DME payment rates effective during SFY 2007 with the following adjustments:

1. Estimate SFY 2007 DME payments for in-state teaching hospitals by applying SFY 2007 DME payment rates to SFY 2007 Nebraska Medicaid inpatient fee-for-service paid claims data. Include all APR-DRG discharges except psychiatric, rehabilitation and Medicaid Capitated Plans discharges.
2. Divide the estimated SFY 2007 DME payments for each hospital by each hospital's number of intern and resident FTEs effective in the Medicare system on October 1, 2006.
3. Multiply the SFY 2007 DME payment per intern and resident FTE by each hospital's number of intern and resident FTEs effective in the Medicare inpatient system on October 1, 2008.
4. Divide the DME payments adjusted for FTEs effective October 1, 2008 by each hospital's number of SFY 2007 claims.
5. Multiply the DME payment rates by the stable DRG budget neutrality factor.

Nebraska hospital-specific DME payment rates are described in state regulations.

On July 1st of each year, the Department will update DME payment rates by replacing each hospital's intern and resident FTEs effective in the Medicare inpatient system on October 1 of the year preceding the beginning of the Nebraska rate year.

Transmittal # NE 24-0011
Supersedes
Transmittal # NE 14-002

Approved January 13, 2025
Effective Date: October 1, 2024

NC FFS GME Methodology with Medicare Comparison

Medicare:

MCR has a payment methodology for the determination of a hospital-specific, base-period per-resident amount (PRA). MCR direct GME payments are calculated using PRA, number of FTE residents, and a hospital's MCR share of total Inpatient days.

$$\text{PRA} = \frac{\text{hospital's allowable costs of GME for base period}}{\text{\# of residents in base period}}$$

Medicare DGME payments = (PRA) x (weighted # FTE residents) x (hospital's MCR share of total IP days)

North Carolina:

North Carolina calculates Medicaid GME payments to all hospitals with Medicare-approved GME programs based on the methodology below. The Medicaid DGME methodology makes use of three similar variables: PRA, FTE residents, and each hospital's share of inpatient days.

$$\text{Statewide PRA} = \frac{\text{total allowable Interns/Residents costs}}{\text{Total \# of FTE residents in base period}}$$

DGME payments = (PRA) x (FTE residents) x (hospital's Medicaid share of hospital IP days)

The steps are as follows:

1. Calculate Statewide PRA: NC calculates a statewide per-resident average (PRA): total allowable intern/resident costs are divided by total FTE residents. The total Interns and Residents Salary and Fringe Costs plus Interns and Residents Other Program Costs use data taken from Medicare Cost Report CMS 2552-10, HCRIS data extract of Worksheet A, Column 7, Lines 21 and 22. The number of resident FTEs is determined from each hospital's HCRIS data in Worksheet S-3, Part 1, Column 9, Line 27.
2. Multiply Statewide PRA by each hospital's number of resident FTEs: FTEs are determined from the hospital's HCRIS data extract from Worksheet S-3, Part 1, Column 9, Line 27 (or from Medicare Year-End Rate Review letter indicating projected number of resident FTEs).
3. Multiply the amount in Step 2 (PRA x hospital FTEs) by each hospital's Medicaid share of inpatient days from HCRIS.

State Plan Under Title XIX of the Social Security Act
Medical Assistance Program

State: NORTH CAROLINA

Payments for Medical and Remedial Care and Services: Inpatient Hospital

GRADUATE MEDICAL EDUCATION (GME) PAYMENT METHODOLOGY

Effective July 1, 2021, the Department of Health and Human Services shall calculate Medicaid GME payments to all hospitals with Medicare-approved GME programs based on the methodology below. All Medicare cost report worksheet, column or line references are based upon the Medicare Cost Report (MCR) CMS 2552 - 10 and should be adjusted for any CMS approved successor Medicare Cost Report (MCR).

- a. The Department shall perform the following calculation to determine direct graduate medical education (DGME) payments to all hospitals with Medicare-approved GME programs except for the University of North Carolina Hospitals dba UNC Hospitals and Pitt County Memorial Hospital, Inc. dba Vidant Medical Center. GME costs and resident counts for the University of North Carolina Hospitals dba UNC Hospitals and Pitt County Memorial Hospital, Inc. dba Vidant Medical Center shall not be included in the calculations described under subpart (a)(i).
 - i. The Direct Graduate Medical Education calculation in this subpart (a)(i) shall be updated annually for each hospital on July 1, based on information from each hospital's Healthcare Cost Report Information System (HCRIS) data most recently filed with CMS and available as of the prior September 30, inflated into the current year by use of the Medicare Hospital Inpatient Prospective Payment System (IPPS) and Long Term Acute Care Hospital (LTCH) Prospective Payment System Final Rule less Productivity Adjustment most recently published in the Federal Register as of July 1 ("market basket update"), as follows:
 1. Calculate a statewide per-resident average (PRA) by:
 - a. Summing the total Interns and Residents Salary and Fringe Costs plus Interns and Residents Other Program Costs as determined from HCRIS data extract of Worksheet A, Column 7, Lines 21 and 22.
 - b. Summing the total number of resident full-time equivalents (FTEs) as determined from each hospital's HCRIS data extract of Worksheet S-3, Part 1, Column 9, Line 27.
 - c. Dividing total allowable direct costs identified in subpart (a)(i)(1)(a) by total resident FTEs identified in subpart (a)(i)(1)(b).

State Plan Under Title XIX of the Social Security Act
Medical Assistance Program

State: NORTH CAROLINA

Payments for Medical and Remedial Care and Services: Inpatient Hospital

2. Multiply the statewide PRA calculated in subpart (a)(i)(1). by each hospital's number of resident FTEs as reported determined by either:
 - a. Each hospital's HCRIS data extract from Worksheet S-3, Part 1, Column 9, Line 27, or
 - b. The hospital's most recent Medicare Year End Rate Review letter dated prior to July 1 and furnished to the Department by September 1, indicating the projected number of IME resident FTEs for the hospital's current fiscal year.
 - (A) The Department shall use each hospital's HCRIS data extract from Worksheet S-3, Part 1, Column 9, Line 27 as a default to determine number of resident FTEs unless the hospital attests that the Medicare Year End Rate Review letter provides a more accurate FTE count.
3. Multiply the amount calculated in subpart (a)(i)(2) by each hospital's Medicaid share of inpatient days as determined from each hospital's HCRIS data as described below. To determine the Medicaid share of inpatient days:
 - a. For hospitals reporting FTEs on Worksheet S-3, Part 1, Column 9, Lines 16 or 17:
 - (A) The numerator shall be the sum of Medicaid (Title XIX) inpatient days from each hospital's HCRIS data extract of Worksheet S-3, Part 1, Column 7, Lines 14, 16, 17, and 32 plus the total number of paid inpatient Medicaid Managed Care days from Worksheet S-3, Part 1, Column 7, Lines 2, 3, and 4.
 - (B) The denominator shall be Total inpatient days from each hospital's HCRIS data extract of Worksheet S-3, Part 1, column 8, Lines 14, 16, 17 and 32.
 - b. For all other hospitals:
 - (A) The numerator shall be the sum of Medicaid (Title XIX) inpatient days from each hospital's HCRIS data extract of Worksheet S-3, Part 1, Column 7, Lines 14, and 32 plus the total number of paid inpatient Medicaid Managed Care days from Worksheet S-3, Part 1, Column 7, Lines 2.
 - (B) The denominator shall be Total inpatient days from each hospital's HCRIS data extract of Worksheet S-3, Part 1, Column 8, Lines 14 and 32.
- b. The Direct Graduate Medical Education calculation in this subpart (b) shall be updated annually for each hospital on July 1, based on information from each hospital's Healthcare Cost Report Information System (HCRIS) data most recently filed with CMS and available as of the prior September 30, inflated into the current year by use of the market basket update, as follows to determine DGME payments to the University of North Carolina Hospitals dba UNC Hospitals and Pitt County Memorial Hospital, Inc. dba Vidant Medical Center.

Summary of Nebraska FFS GME Methodology

Calculation of Medical Education Costs

Direct Medical Education (DME) payments are based on Nebraska hospital-specific DME payment effective rates with the following adjustments:

DGME Calculation

1. Estimate (SFY) DME payments for in-state teaching hospitals by applying (SFY) DME payment rates to (SFY) Nebraska Medicaid inpatient fee-for-service paid claims data. Include all APR-DRG discharges except psychiatric, rehabilitation and Medicaid Capitated Plans discharges.
2. Divide the estimated (SFY) DME payments for each hospital by each hospital's number of intern and resident FTEs effective in the Medicare system on October 1, 2006.
3. Multiply the (SFY) DME payment per intern and resident FTE by each hospital's number of intern and resident FTEs effective in the Medicare inpatient system on October 1, 2008.
4. Divide the DME payments adjusted for FTEs (per effective date) by each hospital's number of (SFY) claims.
5. Multiply the DME payment rates by the stable DRG budget neutrality factor (Link to CMS factor or describe).

Nebraska hospital-specific DME payment rates are found on the states fee schedule

On July 1st of each year, the Department will update DME payment rates by replacing each hospital's intern and resident FTEs effective in the Medicare inpatient system on October 1 of the year preceding the beginning of the Nebraska rate year.

IME Calculation

Indirect medical education (IME) cost payments shall be made to eligible Nebraska teaching hospitals and are calculated by multiplying an IME Factor by the Operating Cost Payment amount.

To calculate annual IME Factor on July 1 of each year, the Department will extract from the CMS Web Pricer Inpatient Prospective Payment System (PPS) the hospital-specific intern-to-bed ratio effective October 1 of the year preceding the beginning of the Nebraska rate year. Hospitals excluded from the Medicare prospective payment system under 42 CFR § 412.23 will have a provider specific intern-to-bed ratio calculated using their Medicare cost report. The intern-to-bed ratio is then utilized in the formula below to calculate the annual IME Factor:

$$[1 + (\text{Number of Interns and Residents} / \text{Available Beds})]^{0.40S_1} * 1.35$$

The IME Factor is not subject to Nebraska Legislative appropriations.

XVII. Graduate Medical Education (GME) Payment.

A. Definitions:

1. Medicare cost report ending in 2018, as reported in Medicare cost reports released on October 19, 2019, with data through September 30, 2019.

2. "Hospital's annualized Medicaid Intern Resident Cost" is the product of the following factors:

a. Annualized intern and resident costs obtained from Worksheet B Part I, Column 21 and 22 the sum of lines 30-43, 50-76, 90-93, 96-98, and 105-112

b. A quotient of:

1. the numerator of which is the hospital's Medicaid days (Worksheet S3 Part I, Column 7, Lines 2-4, 14, 16-18 and 32); and

2. the denominator of which is the hospital's total days (Worksheet S3 Part I, Column 8, Lines 14 and 16-18).

3. "Hospital annualized Medicaid IME payment is the product of the following factors:

a. Hospital IME payments (Worksheet E Part A, Line 29, Coll).

b. A quotient of:

1. the numerator of which is the hospital Medicaid days (Worksheet S3 Part I, Column 7, Lines 2-4, 14, 16-18 and 32); and

2. the denominator of which is the hospital Medicare days (Worksheet S3 Part I, Column 6, Lines 2-4, 14 and 16-18).

4. "Statewide average cost per intern and resident" is the quotient of:

a. The sum of

1. all qualifying hospitals annualized Medicaid Intern Resident Cost

2. all qualifying hospitals annualized Medicaid IME payment

b. the sum of all qualifying hospitals interns and residents as reported on Worksheet S3, Part 1, Col 9, line 14

B. Qualifying Criteria: An Illinois hospital, excluding large public hospitals, reporting intern and resident cost on its Medicare cost report ending in 2018 shall be eligible for a graduate medical education payment.

C. Payment. A qualifying hospital shall receive a payment that is the product of the following factors:

1. the lesser of:

a. the sum of the hospital's annualized Medicaid Intern Resident Cost and annualized Medicaid IME payment, or

b. the product of:

1. the number of interns and residents as reported on Worksheet S3, Part 1, Col 9, line 14, and
 2. 120% of the statewide average cost per intern and resident for all eligible hospitals
2. For payments through December 31, 2022, 22.6 percent.
3. For payments on or after January 1, 2023:
 - a. 35 percent for safety net hospitals or hospitals with 100 or more Full Time Equivalent Residents and Interns, as reported on the hospital's Medicare cost report ending in Calendar year 2018; or
 - b. 30 percent.
4. For payments on or after April 1, 2024, for a children's hospital, as defined in Chapter 11.C.3.a, 100 percent.

STATE PLAN UNDER TITLE XIX OF THE *SOCIAL SECURITY ACT*

State: Illinois

**METHODS AND STANDARDS FOR ESTABLISHING INPATIENT RATES FOR HOSPITAL REIMBURSEMENT;
MEDICAL ASSISTANCE-GRANT (MAG) and MEDICAL ASSISTANCE-NO GRANT (MANG)**

07/18 XVII. Graduate Medical Education (GME) Payment.

A. Definitions:

- 07/20 1. Medicare cost report ending in 2018, as reported in Medicare cost reports released on October 19, 2019, with data through September 30, 2019.
2. "Hospital's annualized Medicaid Intern Resident Cost" is the product of the following factors:
- 07/20 a. Annualized intern and resident costs obtained from Worksheet B Part I, Column 21 and 22 the sum of lines 30-43, 50-76, 90-93, 96-98, and 105-112
- b. A quotient of:
- i. the numerator of which is the hospital's Medicaid days (Worksheet S3 Part I, Column 7, Lines 2-4, 14, 16-18 and 32); and
- ii. the denominator of which is the hospital's total days (Worksheet S3 Part I, Column 8, Lines 14 and 16-18).
3. "Hospital annualized Medicaid IME payment is the product of the following factors:
- a. Hospital IME payments (Worksheet E Part A, Line 29, Col 1).
- 07/20 b. A quotient of:
- i. the numerator of which is the hospital Medicaid days (Worksheet S3 Part I, Column 7, Lines 2-4, 14, 16-18 and 32); and
- ii. the denominator of which is the hospital Medicare days (Worksheet S3 Part I, Column 6, Lines 2-4, 14 and 16-18).
- 07/20 4. "Statewide average cost per intern and resident" is the quotient of:
- a. The sum of
- i. all qualifying hospitals annualized Medicaid Intern Resident Cost
- ii. all qualifying hospitals annualized Medicaid IME payment
- b. the sum of all qualifying hospitals interns and residents as reported on Worksheet S3, Part I, Col 9, line 14
- B. Qualifying Criteria: An Illinois hospital, excluding large public hospitals, reporting intern and resident cost on its Medicare cost report ending in 2018 shall be eligible for a graduate medical education payment.

